Teacher and cover planner

Managing stress and change

Thursday 14 January 2027 · P4 · 40 minutes · Year A

LAUNCH · pair aged 14–16 · Spring 1 Week 2 · formative

Theme Resilience & Change. Six-week term; Week 6 assessing.

Thread tags: SMSC S/So (M/C none); British Values IL/MR (D/RL/T none).

Exact outcome

Managing stress & change (Children's MH Week).

Source

Owner brief, LAUNCH Weekly Spring row 149. Accepted Week 1 supplies structure only. Workbook not reopened. Exact campaign wording is retained in the outcome only.

Purpose and success

Explain stress and possible signs without a diagnosis. Link story clues to a body map or list. Explore gentle breathing, reframing and accessible movement, with safe adaptations, then choose one useful option and support route. Personal plan is private; learning evidence is fictional.

Non specialist cover preparation

Read Staff Answers. Print one tier per learner, Support Cards pages 1–3, glossary and staff Evidence Sheet. Use books/usual aids. Quietly pre-agree the supervised pause route. Use calm fictional examples. Do not create pressure or ask personal symptom questions. Follow usual health, physical access and communication plans. No equipment, internet or special activity is needed.

Forty minute sequence

Phase	Minutes	Action
Arrival	0–4	Calm set-up and discreet supervised pause route.
Starter	4–6	Stress can affect body, thoughts, feelings and actions.
Model	6–11	Teach stress with the invented change; one story check.
Hinge	11–16	Private main and parallel check; reset15–16.
Practice	16–28	Map16–19; breathing19–21; reframe21–24; movement24–27; reset27–28.
Evidence	28–34	Private plan28–30; fictional evidence/selfcheck30–33; reset33–34.
Review	34–38	One NS and actual pupil response.
Exit	38–40	Fictional help route and supported close.

Resets at 15–16, 27–28 and 33–34 are inside the forty minutes. Pair talk is optional and fictional. Adapt, observe or pass is valid for each trial.

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Teaching and response routes

Arrival and starter

0–4: calm set-up, preview and discreet pause route. 4–6: show the invented claim that stress affects only thoughts. Explain that body, feelings and actions can also be involved; no personal examples needed.

Model and checks

6–11: teach meanings using the invented changed-task-order story. Link possible clues without diagnosing. Check what is known versus predicted. 11–15: main hinge B, with parallel A if needed; record a private response. 15–16: reset. Usual aids and the visible model stay.

Map and three trials

16–19: link the three invented clues to the body map or list. 19–21: offer gentle own-pace breathing. 21–24: each learner checks the fictional thought and tries a fairer version in usual mode. 24–27: offer one small familiar movement suited to access and health plans. Show the safe options before each trial. Record tried/adapted/observed/passed/not observed, never a score or effect. 27–28: reset.

Private plan and evidence

28–30: invite a private plan card or fictional/blank/pass route. Keep it with the learner. 30–33: use the short fictional answer already made during practice and learning self-check; do not add a full new written task. 33–34: reset. 34–38: one NS and own reply. 38–40: generic support route and supported close.

Supported

Use short word choices, story clues, pointing or exact scribe/readback. Match three story signs to map/list. Use a fairer-thought choice and one safe strategy choice. All three trials offered; access/adapt/observe/pass remain.

Independent

Use the story to link three signs, explain a fairer thought and choose a safe strategy with a reason and support. All three trials offered. Usual aids stay.

Aspire

Explain why clues are not a diagnosis. Compare two safe approaches to the invented change, test a practical barrier and keep one final choice with support. All three trials offered; extra reasoning never means more personal disclosure.

Worked model and fade

The learner might be responding to the changed plan. Tense shoulders, a fluttery stomach and poor focus are possible signs; they can have other causes. A fairer thought is, “The order changed. I know parts of these tasks. I can ask for the first step.” The learner can try a gentle strategy and ask staff to make the plan clear. For the invented learner: when a familiar task order changes, check a fairer thought and ask school staff for the first step. If that is not useful, ask for another explanation or the agreed supervised pause. Show a model, then leave the choice/reason to the learner. Keep glossary, short frames and usual aids.

TA focus

I do: model non-diagnostic clues and safe trials. We do: offer a story word or frame. Independent: observe the fictional response and actual content prompts, retain usual aids, acknowledge the pause signal and keep supervision. Private card scribing only by agreement; learner keeps it.

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Aids for every task

Each task has a short word bank, starter and model. Choose one tier pack; all retain the same outcome and three trials.

Starter

Words: pressure, change, body, thoughts

Start: Stress can affect…

Model: It may affect focus.

Story and meanings

Words: stress, sign, different, clue

Start: In the story, one clue is…

Model: The learner has tense shoulders.

Private hinge

Words: possible, cause, proof, ask

Start: The clue may mean…

Model: It may be stress, but it is not proof.

Story body map

Words: focus, shoulders, stomach

Start: I link… to…

Model: I link tense shoulders to the shoulder area.

Three safe trials

Words: breath, fairer, move, adapt

Start: A safe choice could be…

Model: Use a gentle move within the usual access plan.

Plan and self check

Words: cue, choice, support, alternative

Start: For the story learner…

Model: Ask staff to show the first step.

Review and help

Words: reason, next step, trusted adult

Start: My story answer now shows…

Model: A fairer thought fits what is known.

Prompt ladder

Point to the place or instruction; ask a short private question; rephrase; offer a choice or scaffold; model the first step only, then return the task. Allow processing time. Record content prompts separately from usual reading, language and communication access.

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Checks and planned access

Stress and clues

Stress is a response to pressure or change. It can affect body, thoughts, feelings and actions. Signs vary and can have other causes; the story map cannot diagnose. Reframing checks known facts and tries a fairer view, including real support.

Main hinge and decision

What can the story clues tell us? A: They prove the learner has a condition. B: They may show pressure, but can have other causes. Correct B. If A is chosen, explain that signs alone cannot establish a condition; re-model then use the parallel privately. No public answer round.

Parallel hinge

Which response fits the invented change? A: Check the facts and ask staff to explain the first step. B: Tell the learner to ignore every concern. Correct A. A fairer view can include real support. If unclear, keep the model and revisit later; do not label the learner.

Cognition and Learning

Explicit short definitions, worked example, glossary, visible model, task frames and one chosen step. Flexible grouping for a pair; reduced written load and extra processing time.

Communication and Interaction

Choice, wait time, familiar aid, pointing/drawing, exact scribe and readback. Pair discussion optional; no requirement for speech or eye contact.

SEMH

Predict the sequence, keep all stories invented and mild, offer a quiet supervised pause and supported return. No public feeling check. Story body map and private plan can be blank, fictional or passed; never collect personal details.

Sensory/Physical

Keep the room calm and the usual access and health plans. No induced pressure or noise. Offer one small familiar movement, adapted to access and comfortable range; observing or passing is equal. Breathing stays gentle at own pace, with no holds. Stop discomfort and seek staff help.

EAL

Pre-teach stress, sign and reframing with short plain meanings and the invented story. Allow planning in any language, pointing, glossary and usual language support. No personal explanation is required.

Optional private story checks

F: Point to one possible story clue. M: Explain a fairer thought using the known facts. H: Adapt a strategy if the learner cannot or does not wish to use it. No current-feeling check or public answer round.

Misconceptions

A body clue proves stress. Clues vary and can have other causes; no classroom diagnosis.

A strategy must work for everyone. Access, context and preferences vary. Adapt or ask for support.

Reframing means pretending all is fine. A fairer thought fits facts and can include a real problem and help.

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Safe trials and private reflection

Gentle breathing

Sit or stand in your usual supported position. Let an easy breath come in and out at your own pace. Keep breathing comfortable. No hold, forced deep breath, set rate or count. Try briefly, then return to usual breathing. Eyes can stay open. Stop and tell staff if uncomfortable or dizzy. You may observe, use normal breathing or pass.

Reframing

Use the invented thought: “I will get every part wrong.” What is known? The order changed, but the tasks are familiar. Try: “I know some parts. I can ask for the first step.” Say, point to, type or write the fairer thought. A fairer thought need not be cheerful. It must fit the facts. Do not reframe harm as acceptable or dismiss a real problem. Asking for a clearer plan is valid.

Accessible movement

With staff agreement, try one small, familiar move that suits your access plan: a gentle hand move, a seated stretch or a few steps in the agreed clear space. Stay in a comfortable range. No touch, exertion, balance challenge or performance target. Use a familiar alternative or observe/pass. Stop if painful or uncomfortable; follow usual health and access plans.

Private card and self check

Choose one possible strategy for a mild change, privately, or plan for the invented learner. Include practical support. No personal worry, symptom, health or family detail is needed. Keep the card. Blank or pass is valid. One useful choice and a support route. No promise to feel calm and no requirement to cope alone. Strategies do not replace help with the cause of pressure. All boxes start blank. This checks learning, not feelings, symptoms or whether a strategy worked. Observing, adapting or passing is recorded fairly, without a health judgement.

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Privacy support and evidence

Discreet supervised pause

Before the lesson, quietly agree the familiar pause signal and supervised place/route with the learner. A pupil may show a pause card or use the usual aid without a public explanation. A staff member acknowledges and supports the agreed route; never send a learner off alone or remove supervision. Offer a neutral task and a supported return, or arrange a later private catch-up. Pass/step-out is not failure and does not require personal disclosure.

Safeguarding and privacy

Any disclosure goes to the safeguarding lead through school procedure. Listen calmly, reassure, do not probe, investigate or promise secrecy. Follow urgent school procedures where needed. Keep actual concerns and identifying details out of the lesson pack, book tasks and shared learning evidence; use the school's separate safeguarding record. If a learner cannot identify a trusted adult, support them privately and do not press. Private plans are not collected, photographed, displayed or compared for assessment. Scribing the personal plan is only by agreement; the learner keeps it.

Help by type

Support may come from a trusted adult, school staff, a family doctor or a suitable helpline. Ask trusted school staff for help to find an age-appropriate route. Seek help if a concern persists, worsens, affects everyday life or feels unsafe; you do not need a diagnosis first. If the first adult is unavailable or unhelpful, try another trusted adult through the agreed route. For immediate concern, get school staff straight away and use the school's urgent procedure.

Evidence and marking

Record explanation of stress, three fictional body-map links, a fairer thought, safe strategy knowledge and a generic support route. Record each breathing/reframing/movement opportunity as tried, adapted, observed, passed or not observed; no pass is failure. Do not record symptoms, feelings, private plan content or claimed improvement. Distinguish usual access from actual adult content prompts. Use a work code, one NS and actual pupil response. E requires saved actual work; R requires Voice and Audience plus NS and own response. VF verbal feedback; WS worked with support; I independent; NS next step in green; // self-edit; ? read this back to me; E actual evidence banked; R responded. R only when Voice and Audience are present, one NS is set and the pupil responds in the same colour, on the same page or usual mode. Offer to the Communication portfolio; staff confirm any mapping.

Where this leads

Next learning applies calm decision-making and help-seeking to safe choices online. Offer fictional learning to the Communication portfolio; staff confirm mapping and evidence sufficiency.

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